

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

February 8, 2024

MEMORANDUM FOR:

(b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

(b)(6),(b)(7)(C),(b)(7)(F) MD
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

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Chief of Staff
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CDR (b)(6),(b)(7)(C),(b)(7)(F), RN
Chief, Investigations Unit
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FROM:

LCDR (b)(6),(b)(7)(C),(b)(7)(F), DNP, RN
Senior Investigator, Investigations Unit
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

SUBJECT:

Mortality Review – Report of Findings
Carlos Juan FRANCISCO, (b)(6),(b)(7)(C)

Executive Summary:

On December 4, 2023, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of noncitizen Carlos Juan FRANCISCO, (b)(6),(b)(7)(C). (b)(6),(b)(7)(C) ICE assigned Mr. FRANCISCO, a 42-year-old Guatemalan male, to Krome Service Processing Center (KSPC), in Miami, Florida (FL), from November 23 to December 4, 2023, the date of his death. Mr. FRANCISCO's death certificate and autopsy report are pending.

On an unknown date and place, Mr. FRANCISCO entered the United States without an immigration official inspection or parole. Between January 4, 1999 and October 9, 2023, the Twentieth Judicial Circuit Court of Lee County, located in Ft. Myers, FL, convicted Mr.

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FRANCISCO with crimes ranging from burglary to aggravated assault with a deadly weapon, and sentenced him to a combined nonconsecutive incarceration equaling approximately 40 months at Lee County Jail (LCJ). Prior to Mr. FRANCISCO's completion of his incarceration, ICE Enforcement and Removal Operations (ERO) charged him with inadmissibility pursuant to Section 212(a)(6)(A)(i) of the Immigration and Nationality Act (INA), as a noncitizen who entered the United States without inspection, Section 212(a)(7)(A)(i)(I) of the INA, as an immigrant without a valid immigrant visa, and 212(a)(2)(A)(i)(I) of the INA, as a noncitizen who has been convicted of a crime involving moral turpitude. On November 22, 2023, LCJ officials released Mr. FRANCISCO to ERO Miami, and ERO detained Mr. FRANCISCO at KSPC.

On November 23, 2023, during his intake screening, Mr. FRANCISCO denied history of any medical or mental health conditions, illicit drug use, or hospitalizations, and health staff cleared him for general population (GP).

Between November 29 and December 1, 2023, KSPC's health staff evaluated and treated Mr. FRANCISCO for headaches.

On December 2, 2023, Mr. FRANCISCO vomited his breakfast and lunch and complained of headaches. The health staff admitted Mr. FRANCISCO to the medical housing unit (MHU) for monitoring, administered one round of intramuscular anti-nausea medicine, and ordered a liquid diet. While in the MHU, Mr. FRANCISCO vomited once, and the attending advanced practice provider (APP) referred Mr. FRANCISCO to a local emergency room (ER) via ambulance for dehydration evaluation. Approximately 45 minutes later, responding emergency medical services (EMS) personnel assumed Mr. FRANCISCO's care and transported him to Larkin Community Hospital (LCH) ER, where an ER physician diagnosed Mr. FRANCISCO with atraumatic subdural hematoma (spontaneous bleeding between the brain and its outmost covering). The hospital team intubated (a procedure involving the passage of a tube into the trachea to help a person breathe) Mr. FRANCISCO, and a neurosurgeon performed evacuation of his hematoma (removal of blood from the brain).

On December 3, 2023, a repeat scan of Mr. FRANCISCO's head revealed bleeding in the brain stem; the neurosurgeon did not pursue any further intervention. On December 4, 2023, Mr. FRANCISCO went into cardiac arrest and an LCH physician declared him deceased.

Mortality Finding:

As a result of Mr. FRANCISCO's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

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A mortality review committee (MRC) determined the care KSPC provided Mr. FRANCISCO deviated beyond safe limits of practice and may have indirectly contributed to his death. Specifically, the MRC concluded :1) Health staff should have consulted/escalated Mr. FRANCISCO's case with and to the clinical director for input; 2) Health staff delayed Mr. FRANCISCO's referral to the ER; 3) Health staff did not immediately recognize potential acute on chronic brain bleeding symptomology; 4) Health staff chose contracted ambulance service over 911 for Mr. FRANCISCO's ER referral; and 5) Custody staff did not consistently consult health staff on issues pertaining to Mr. FRANCISCO's health.

The MRC convened on February 5, 2024, and consisted of the following IHSC members: Dr. (b)(6),(b)(7)(C),(b)(7)(F) clinical director, physician reviewer; Lieutenant (LT) (b)(6),(b)(7)(C),(b)(7)(F) physician assistant, APP reviewer; Captain (b)(6),(b)(7)(C),(b)(7)(F) IHSC chief nurse, nurse reviewer; and LT (b)(6),(b)(7)(C),(b)(7)(F) risk manager.

The following summary of events was based on: 1) Mr. FRANCISCO's KSPC medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. FRANCISCO's KSPC detention file; 5) video footage dated December 2, 2023; 6) an on-site review and staff interviews conducted by Lieutenant Commander (b)(6),(b)(7)(C),(b)(7)(F) at KSPC from January 3 to 4, 2024; and 7) applicable IHSC policies and procedures, and ICE Detention Standards.

Summary of Events

On November 23, 2023, a registered nurse (RN) completed Mr. FRANCISCO's intake screening, documented normal vital signs, Mr. FRANCISCO's denial of any past medical or mental health history, and cleared him for GP. Mr. FRANCISCO's Collier County (CC) transfer summary showed no medical issues identified while in CC custody.

On November 27, 2023, during sick call, an RN evaluated Mr. FRANCISCO for a complaint of headaches for two days, rated at 5 out of 10 [(5/10); based on a subjective pain scale with a rating of 10 equaling severe]. The RN documented a normal assessment and vital signs, provided Mr. FRANCISCO a one-time dose of ibuprofen (pain medicine) 200 milligrams (mg), two tablets, per clinical nursing guidelines (CNG), and cleared him for GP.

On November 29, 2023, during sick call, an RN evaluated Mr. FRANCISCO for a complaint of a 5/10 headache, documented a normal assessment and vital signs, and described Mr. FRANCISCO's appearance as uncomfortable due to pain. The RN provided Mr. FRANCISCO a one-time dose of acetaminophen (pain medicine) 325 mg, two tablets, per CNG, monitored him for 30 minutes, and cleared him for GP following his report of feeling better.

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- On the same date, at 10:08 a.m., a custody staff member announced a medical emergency/wheelchair in alpha yard via radio.
- At 10:12 a.m., responding health staff arrived at the scene and transported Mr. FRANCISCO to the medical clinic where an APP urgently evaluated him for a complaint of an 8/10 throbbing headache located on the right side of his head and radiating to his right eye. The APP documented Mr. FRANCISCO reported experiencing headaches, relieved with pain medicine, months before his detention. The APP noted a normal assessment and vital signs, and ordered aspirin-acetaminophen-caffeine (migraine headache medicine), 250-250-65 mg, two tablets, by mouth, daily, as needed (PRN), and ibuprofen 800 mg tablet, twice daily PRN. The APP provided Mr. FRANCISCO a dose of ibuprofen and an ice pack, monitored him for an hour, and cleared him for GP after his report of no headaches.

On December 2, 2023, at approximately 8:06 a.m., a custody staff member announced a medical emergency/wheelchair at the cafeteria via radio. Approximately three minutes later, health staff arrived at the cafeteria and transported Mr. FRANCISCO to the medical clinic.

- At approximately 8:09 a.m., an APP evaluated Mr. FRANCISCO for complaints of an 8/10 headache, body aches, and nausea with vomiting, and documented normal vital signs. Mr. FRANCISCO reported intermittent headaches for the past two months, now described as frequent for the past four to five days, and denied head trauma. The APP noted Mr. FRANCISCO was awake, alert, and oriented, and provided him a lower bunk pass, a one-time dose of acetaminophen 650 mg tablet, and an antacid. The APP encouraged Mr. FRANCISCO to increase his fluid intake, monitored him for approximately one hour, and cleared him for GP due to his reported improvement of his nausea and headache.
- At approximately 12:45 p.m., an APP evaluated Mr. FRANCISCO for complaints of an 8/10 headache, nausea, and vomiting. The APP documented normal vital signs, except a low heart rate of 55 beats per minute [(bpm); normal: 60-99 bpm], ordered a clear liquid diet and a one-time dose of intramuscular ondansetron (nausea medicine) 4 mg (administered via the left deltoid muscle), and admitted Mr. FRANCISCO to the MHU for close monitoring.
- At approximately 4:19 p.m., an RN noted Mr. FRANCISCO vomited his lunch and dinner and complained of epigastric pain. The RN documented normal vital signs, except an elevated blood pressure of 143/73 millimeters of mercury [(mmHg); normal: less than 120/80] and a slow heart rate of 55 bpm. The RN noted an APP referred Mr. FRANCISCO to LCH for a higher-level evaluation via ambulance.

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- On the same date, an LCH physician diagnosed Mr. FRANCISCO with atraumatic subdural hematoma and consulted a neurosurgeon who sedated, intubated, performed a craniotomy (surgical opening of a part of the skull) with hematoma evacuation on Mr. FRANCISCO, and admitted him to the intensive care unit.

On December 3, 2023, an LCH physician documented follow-up imaging showed new bleeding in the brain stem and consulted with a neurosurgeon who did not pursue any further intervention due to worsening of the bleed.

On December 4, 2023, an LCH physician pronounced Mr. FRANCISCO deceased.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, National Commission on Correctional Health Care (NCHC) Standards for Health Services in Jails, 2018, and IHSC Directives. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) Nursing staff provided Mr. FRANCISCO timely medical care using CNG; and 2) Health staff responded to the medical emergencies within four minutes.

Weaknesses

1. Inappropriate Medical Care

- On November 29, 2023, an APP provided Mr. FRANCISCO ibuprofen (the medication may increase the risk of bleeding) and prescribed another medication containing aspirin (blood thinner). Later that evening, an RN administered ibuprofen and aspirin-acetaminophen-caffeine simultaneously to Mr. FRANCISCO, increasing his risk of bleeding.
- The APP who evaluated Mr. FRANCISCO on December 2, 2023, did not consider a differential diagnosis of acute on chronic brain bleed, nor did the APP recognize the symptomology despite Mr. FRANCISCO's recurrent complaints of headaches coupled with weakness, nausea, and vomiting. As a result, the APP delayed Mr. FRANCISCO's higher-level referral to the ER.
- The APP responsible for Mr. FRANCISCO's care on December 2, 2023, initially cleared Mr. FRANCISCO for GP after evaluating him for vomiting coupled with weakness and headaches. The APP did not seek the KSPC clinical director's input despite Mr. FRANCISCO's symptoms and the number of times the health staff evaluated him for headaches.

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- The APP who referred Mr. FRANCISCO to the ER chose a contracted ambulance service (estimated arrival time varies between one to two hours) over calling 911 (EMS personnel estimated arrival time less than one hour) because the APP did not consider Mr. FRANCISCO's condition high in acuity.
- On December 2, 2023, health staff were not consistently within sight and sound of Mr. FRANCISCO following his admission to MHU.
- On November 29, 2023, after becoming aware of Mr. FRANCISCO's vomiting episode, health staff did not triage and evaluate him nor pursue obtaining his signature on a refusal form.

Applicable standards of care for this finding:

- IHSC Directive: 02-07: Treatment Consent and Refusal Policy, 2022; section 6-7: *Refusal of Treatment or Health Evaluation*.
- IHSC Directive: 03-02: Access to Care: Sick Call, 2021; section 5: *Procedures*.
- IHSC Directive: 03-17: Medical Housing Units, 2016; section 4-8: *MHU Safety and Security*.
- IHSC Directive: 02-07: Treatment Consent and Refusal Policy, 2022; section 6-7: *Refusal of Treatment or Health Evaluation*.
- NCCHC: Standards for Health Services in Jails, 2018: J-B-08, *Patient Safety*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-09, *Continuity Coordination, And Quality of Care During Incarceration*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-10, *Discharge Planning*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; subsection II. *Expected Outcomes*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; subsection F. *Facilities/Medical Housing*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; subsection AA. *Informed Consent and Involuntary Treatment*.

2. Communication

- On December 2, 2023, an APP ordered a liquid diet for Mr. FRANCISCO due to repeated vomiting episodes. An RN communicated the diet order to custody staff; however, a custody staff member provided Mr. FRANCISCO with a brown bag containing fruits. Immediately after eating the fruits, Mr. FRANCISCO began vomiting.
- On December 2, 2023, custody staff observed Mr. FRANCISCO in a confused state but did not report Mr. FRANCISCO's abnormal mental state to health staff.

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- On December 2, 2023, a custody supervisor coordinated Mr. FRANCISCO's transfer from MHU to KSPC processing area without the health staff's prior authorization, consultation, and concurrence.
- On December 2, 2023, health staff were absent to exchange Mr. FRANCISCO's medically relevant information with responding EMS personnel. As a result, EMS personnel assumed Mr. FRANCISCO's care without health staff input.
- On November 29, 2023, custody staff did not obtain health staff concurrence before allowing Mr. FRANCISCO, who vomited, to return to his dorm without health staff evaluation.

Applicable standards of care for this finding:

- IHSC Directive: 03-02: Access to Care: Sick Call, 2021; section 5: *Procedures*.
- IHSC Directive: 03-17: Medical Housing Units, 2016; section 4-8: *MHU Safety and Security*.
- IHSC Directive: 02-07: Treatment Consent and Refusal Policy, 2022; section 6-7: *Refusal of Treatment or Health Evaluation*.
- NCCHC: Standards for Health Services in Jails, 2018: J-D-06, *Patient Escort*.
- NCCHC: Standards for Health Services in Jails, 2018: J-B-08, *Patient Safety*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-09, *Continuity Coordination, And Quality of Care During Incarceration*.
- NCCHC: Standards for Health Services in Jails, 2018: J-E-07, *Nonemergency Health Care Requests and Services*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; subsection W. *Special Needs and Close Medical Supervision*.
- PBNDS 2011 with 2016 Revisions: Part 2.9, Post Orders; subsection B. *Reading and Understanding of Post Orders*.
- PBNDS 2011 with 2016 Revisions: Part 4.8, Disability Identification, Assessment, and Accommodation; subsection F. *Reasonable Accommodation Process/Immediate Accommodation*.

3. Inappropriate Emergency Care

- On December 2, 2023, health staff responded to the scene of a medical emergency without the required medical emergency bag.

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Applicable standards of care for this finding:

- IHSC Directive: 03-05: All Hazards Emergency Preparedness, 2016; section 4-12: *Medical Equipment and Supplies Management*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.

4. Training

- A currently employed custody staff member has an expired basic life support card.

Applicable standards of care for this finding:

- NCCHC: Standards for Health Services in Jails, 2018: J-C-04, *Health Training for Correctional Officers*.
- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid*.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director (DAD) of Health Systems Support (HSS).
- The IHSC DAD of HSS will share these findings through appropriate communication channels to ICE, KSPC's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HSS unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.